Can people with disabilities actually get there?

A simple way to check the trip to a hospital or research site, by transit, car or ride. Page 1: the route.

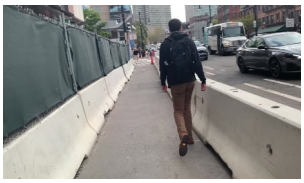
How it works

1. Walk the route from the transit stop or drop-off to the front desk, recording video and talking through what you see.
2. List every barrier: where it is, who it affects, and a suggested fix.
3. Share the list with the site, the city and the transit agency, and check later whether it was fixed.

Examples from the Boston area (faces blurred)



Subway entrance with no elevator, and no sign to the one across the square



Temporary walkway squeezed between barriers and traffic near a hospital



Bumpy warning strip for blind pedestrians, peeled and broken



2-inch gap between sidewalk and street: can stop a wheelchair wheel

Route checklist: try it on your own route

Done	Item	What to check
☐	**Transit stop**	Elevator at this entrance, and working? Clean, lit and easy to see from the street? How many elevators on the whole trip, including transfers?
☐	**Signs**	A sign to the step-free route where people choose their way, not only at the barrier? Simple words and symbols?
☐	**Distance and rest**	How far between seats? Seats wherever people wait: at the stop, in line, at check-in? Shade or shelter? Restrooms along the way?
☐	**Surface and slope**	Brick, cracks, gaps, tree roots, ice, puddles? Stairs, hills, bumpy stretches? Railings or something to hold?
☐	**Crossings**	Curb ramp lined up with the crosswalk? Bumpy warning strip at the curb? A signal that beeps or talks? Walk time long enough for a slow walker? (Time it.)
☐	**Drop-off and parking**	Loading zone for a wheelchair van? Accessible parking near the door? Can anyone who asks be dropped at the door, placard or not?
☐	**Entrance**	Automatic door? Button within reach? Heavy doors? Sign to the accessible door?
☐	**Inside**	Distance to the desk, desk height, check-in kiosk usable without sight, elevator, restroom, seating, a place to lie down?
☐	**Senses**	Lighting at night, glare, noise, crowds, smoke, smells? A quiet place to wait?
☐	**The day itself**	Construction detours: step-free and signed? Snow and ice on ramps? Your ride's pickup time?

Independent project by Uri Kartoun, DBbun LLC (dbbun.com), entered in the NIH challenge on including people with disabilities in clinical research. Not NIH-funded. Prepared with AI assistance (Claude). Free to use and adapt with credit (CC BY 4.0): github.com/DBbun/route-to-research-checklist

Plan the visit so people can get there

Page 2: for study teams and research offices. Ask each participant what they need; don't require proof or a placard.

Many barriers can't be seen on a walk, and only the study team can fix them. Most of these items came from people with disabilities and chronic illnesses who commented on earlier versions.

Visit checklist

Done	Item	What to check
☐	**Directions**	Send step-free directions with the visit invitation: nearest accessible station entrance, drop-off point, accessible door, where to check in. Use plain language and a photo of each step.
☐	**Timing**	Schedule visits to fit the train or bus timetable. Count waiting time in the door-to-door total. Offer the first or last slot of the day to avoid crowds.
☐	**Ride**	Can the study book and pay for the ride directly, so the participant pays nothing up front and no wait-time fees? Allow for disability van services (paratransit), whose pickup times aren't exact.
☐	**Cost**	Reimburse fares, parking and rides, and say how much per visit. Acknowledge lost income and recovery time, and state the unit (per visit or per year).
☐	**Companion**	Can someone come along to help? Does the study cover their travel too?
☐	**Communication**	Can people check in and reach the study team by text or email, not only by phone? Sign language interpreter booked ahead? Staff ready for people who speak with a device, a letter board or pictures, or who need more time?
☐	**Rest**	A place to lie down before and after the visit. Seats wherever people wait. A restroom near the visit room.
☐	**Fewer trips**	Which visits could be done by phone or video, or at home? Ask people to come in only when a test or procedure needs it.
☐	**Ask**	Ask every participant what would make the trip possible, and offer the same help whether or not their disability is visible.

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